



Statement of Issues

7 November 2024

Blackstone Group (HK) Limited – proposed acquisition of I'rom Group Co Limited

Purpose

1. Funds managed or advised by Blackstone Group (HK) Limited or its related bodies corporate (**Blackstone**) propose to acquire all of the shares of I'rom Group Co. Limited (**I'rom**) (the **proposed acquisition**). Blackstone, through related bodies corporate, indirectly owns Nucleus Network Pty Limited (**Nucleus**) and I'rom owns CMAX Clinical Research Pty Ltd (**CMAX**).
2. This Statement of Issues:
 - gives the Australian Competition and Consumer Commission's (**ACCC**) preliminary views on competition issues arising from the proposed acquisition
 - identifies areas of further inquiry, and
 - invites interested parties to submit comments and information to assist the ACCC's assessment of the issues.
3. Statements of Issues do not refer to confidential information provided by the parties or other market participants and therefore may not fully articulate the ACCC's preliminary position.

Overview of ACCC's preliminary views

4. In considering the proposed acquisition, the ACCC applies the legal test set out in section 50 of the *Competition and Consumer Act 2010* (the **CCA**). In general terms, section 50 prohibits acquisitions that would have the effect, or be likely to have the effect, of substantially lessening competition in any market.
5. The ACCC divides its preliminary views into three categories, 'issues of concern', 'issues that may raise concerns' and 'issues unlikely to raise concerns'. In this Statement of Issues there is one 'issue of concern' and one 'issue that may raise concerns'.

Issue of concern

6. The ACCC's preliminary view is that the proposed acquisition is likely to have the effect of substantially lessening competition in the supply of phase 1 clinical trial services in

Australia. The proposed acquisition brings the two largest providers of phase 1 clinical trial services in Australia under common ownership and is likely to significantly lessen the competitive constraints on these providers.

7. The ACCC is concerned that this is likely to lead to the prices of phase 1 clinical trial services in Australia being higher and/or the quality of those services being lower, than they would be absent the proposed acquisition.

Issue that may raise concerns

8. The ACCC is concerned that the proposed acquisition may also have the effect of substantially lessening competition in the supply of services offered by contract research organisations (**CROs**). CROs provide services to drug sponsors and assist with selecting phase 1 clinical trial service providers.
9. Blackstone has a majority interest in Precision Medicine Group, which operates the Precision for Medicine CRO. Precision for Medicine supplies clinical trial monitoring and management services to pharmaceutical and biotechnology companies, including in Australia. The ACCC is concerned that post acquisition Blackstone may have the ability and incentive to foreclose CROs which compete with Precision for Medicine by restricting their access to phase 1 clinical trial services provided by Nucleus and CMAX. Such conduct may significantly lessen competition in the supply of CRO services in Australia.

Making a submission

10. The ACCC invites submissions from interested parties, particularly on the following key issues:
 - the closeness of competition between Nucleus and CMAX, including in relation to quality, pricing and capacity to undertake new trials quickly
 - the extent to which other Australian providers of phase 1 clinical trial services compete with Nucleus and CMAX, including in relation to quality, pricing and capacity to undertake new trials quickly
 - the extent to which internationally based providers of phase 1 clinical trial services compete with Nucleus and CMAX for customers in Australia including in relation to quality, pricing, capacity to undertake new trials quickly and regulatory approval systems
 - the height of barriers and likelihood of timely and effective new entry and/or expansion by alternative providers of phase 1 clinical trial providers in Australia
 - the likelihood the proposed acquisition would result in prices of phase 1 clinical trial services being higher and/or the quality of those services being lower than they would be absent the proposed acquisition, and
 - whether rival CROs would have access to competitively effective alternative providers of phase 1 clinical trial services if they were unable to access services provided by Nucleus and CMAX.
11. Interested parties should provide submissions by 5pm on 21 November 2024. Responses may be emailed to mergers@accc.gov.au with the title: *Submission re: Blackstone's proposed acquisition of I'rom*.

12. The ACCC anticipates making a final decision on 6 February 2025, however, this timeline can change. To keep up with possible timing changes and to find relevant documents, interested parties should visit the Mergers Register on the ACCC's website at www.accc.gov.au/publicregisters/mergers-registers/public-informal-merger-reviews.

Confidentiality of submissions

13. The ACCC will not publish submissions regarding the proposed acquisition. The ACCC will not disclose submissions to third parties (except our advisors/consultants) unless compelled by law (for example, under freedom of information legislation or during court proceedings) or in accordance with s155AAA of the CCA. Where the ACCC is required to disclose confidential information, the ACCC will notify you in advance where possible so that you may have an opportunity to be heard. Therefore, please identify any confidential information that is provided to the ACCC. The ACCC's [Informal Merger Review Process Guidelines](#) contain more information on confidentiality.

About ACCC 'Statements of Issues'

14. A Statement of Issues is not a final decision about a proposed acquisition. A Statement of Issues outlines the ACCC's preliminary views and identifies further lines of inquiry.
15. A Statement of Issues provides an opportunity for all interested parties (including customers, competitors, shareholders and other stakeholders) to ascertain and consider the primary issues identified by the ACCC. It is also intended to provide the merger parties and other interested parties with the basis for making further submissions should they consider it necessary.

The parties

The acquirer: Blackstone

16. Blackstone is one of the largest alternative asset managers operating globally.
17. Relevant to the proposed acquisition, Blackstone indirectly owns Nucleus. Blackstone also has a majority interest in Precision Medicine Group, which operates the Precision for Medicine contract research organisation (**CRO**). Precision for Medicine supplies clinical trial monitoring and management services to pharmaceutical and biotechnology companies and has operations in North America, Europe, Asia and Australia. However, the Australian operations are currently limited.
18. Nucleus supplies phase 1 clinical trial services to biotechnology and pharmaceutical companies, based both in Australia and internationally. In Australia, Nucleus currently operates clinical units in Brisbane, Queensland and Melbourne, Victoria, as well as a satellite screening and outpatient clinic in Geelong, Victoria. Nucleus has 94 beds at its Melbourne clinic and 60 beds at its Brisbane facility. Nucleus has also announced that it intends to open a phase 1 clinical trial unit in Sydney.

The target: I'rom

19. I'rom is a healthcare business based in Japan which specialises in the research and development of medicine and gene technology. I'rom provides services to medical institutions conducting clinical trials, including site management and CRO services.

20. In Australia, I'rom supplies phase 1 clinical trials through CMAX, and, to a limited extent, clinical trial services for other research phases through Fusion Clinical Research Pty Ltd (**Fusion**). Fusion administers late-stage clinical trials, which do not overlap with services provided by Nucleus and CMAX.
21. CMAX supplies phase 1 clinical trial services to biotechnology and pharmaceutical companies, in Australia and internationally. CMAX currently operates one clinical unit in Adelaide, South Australia, which has 78 beds.

Industry Background

22. Clinical trials are a step in the regulatory approval of new therapeutic products such as medicines and medical devices. Clinical trials are generally conducted in phases with each phase having a different focus.
23. Phase 1 clinical trials involve medicine being administered in low doses to humans for the first time. Phase 1 clinical trials are generally conducted with healthy volunteers and are focused on testing the safety and tolerability of medicine.
24. The number of healthy volunteers required for a phase 1 clinical trial depends on the medication and type of trial being undertaken. Market participants indicated that, for typical phase 1 clinical trials, between 20 and 80 healthy volunteers are generally appropriate, although there are likely to be separate cohorts dosed over a number of weeks.
25. In some limited circumstances, such as where the intervention is known to carry significant health side-effects, phase 1 clinical trials may be conducted using a cohort of patients rather than healthy volunteers. This is common for phase 1 clinical trials related to oncology medications.
26. Phase 1 clinical trials involving healthy volunteers are generally conducted in purpose-built clinical trial facilities. These facilities usually have an inpatient bed capacity, so that volunteers can be contained and monitored closely for a period after medication is administered. These facilities are equipped similarly to hospitals and are staffed by medical professionals with experience managing clinical trials.
27. If a phase 1 clinical trial indicates that a new product is safe, the product can then proceed to phase 2, 3 and 4 trials where it is tested more extensively, including for efficacy. Data from phase 1 clinical trials conducted in Australia can be used to conduct later phase clinical trials in other jurisdictions.

Australia's regulatory environment

28. Market participants indicated that Australia is generally considered to have a favourable regulatory pathway for phase 1 clinical trials. The Clinical Trial Notification (**CTN**) scheme in Australia requires that drug sponsors wishing to carry out clinical trials in Australia obtain ethical approval from a Human Research Ethics Committee (**HREC**) and then notify the Therapeutic Goods Administration (**TGA**) of their intent to carry out the trial. Trials are able to commence once they have been notified to the TGA without any formal approval being required.
29. Market participants have indicated that the CTN scheme in Australia can enable trials to commence significantly faster than in other jurisdictions. Market participants

indicated that trials may be able to be commenced within 6 weeks of making an application for HREC approval.

30. In many other jurisdictions, the regulatory pathway is significantly more onerous. For example, in the USA, sponsors must receive approval from a HREC and complete an Investigation of New Drug (**IND**) application, which is assessed by the Food and Drug Administration. Market participants indicated that receiving IND approval can take up to 6 months.
31. Data collected from phase 1 clinical trials conducted in Australia is considered to be high quality and is accepted by authorities in all major jurisdictions (which is beneficial if the drug sponsor wishes to continue conducting later phase clinical trials in an alternative jurisdiction). Many sponsors value the ability to conduct their phase 1 trial relatively quickly in Australia, then use the data to conduct subsequent trials in other jurisdictions.

Australian R&D Tax Incentive

32. Australia offers a research and development tax incentive program (**R&D Tax Incentive**) which allows companies to claim tax offsets on eligible research and development activities.¹
33. Companies carrying out eligible activities in Australia may be eligible for tax offsets in Australia, which vary based on the entities' turnover and expenditure.²
34. Market participants have indicated that phase 1 clinical trials are generally considered to be an eligible research and development activity. As such, drug sponsors undertaking phase 1 clinical trials in Australia may be able to access the R&D Tax Incentive, which provides a strong incentive for sponsors to carry out their R&D activities in Australia.

Current competitive landscape for phase 1 clinical trial services

35. Nucleus and CMAX are the two largest providers of phase 1 clinical trial services in Australia. Nucleus has 94 beds at its Melbourne unit and 60 beds at its Brisbane unit and CMAX has 78 beds at its Adelaide unit. Other providers have significantly smaller footprints and less capacity than both Nucleus and CMAX as shown in **Table 1** below.

Table 1: Bed numbers of Australian phase 1 clinical trial service providers

Provider	Location	Number of beds	Share of beds in Australia
Nucleus	Melbourne, Brisbane	154	39.4%
CMAX	Adelaide	78	19.9%
Merged entity	Melbourne, Brisbane, Adelaide	232	59.3%

¹ Australian Taxation Office, [Eligibility for R&D tax offsets](#), accessed October 2024.

² Australian Taxation Office, [Rates of R&D tax incentive offset](#), accessed October 2024.

Provider	Location	Number of beds	Share of beds in Australia
Veritus	Melbourne	42	10.7%
Linear	Perth	32	8.2%
Scientia	Sydney	30	7.7%
Novatrials	Newcastle	30	7.7%
Doherty Clinical Trials	Melbourne	25	6.4%
University of the Sunshine Coast	Sunshine Coast, Moreton Bay and Brisbane	Not publicly available	Unknown
Total		391	100%

36. The ACCC considers that bed numbers may be representative of the general capacity of phase 1 clinical trial service providers, but notes specialist skills and experience are required to successfully deliver phase 1 clinical trial services. These requirements are discussed in more detail at paragraphs 58-63 below. The ACCC is also considering other measures of the share of supply in Australia including the number of trials undertaken and revenue earned by providers for phase 1 clinical trials.
37. Linear Clinical Research (**Linear**) in Perth and Scientia Clinical Research (**Scientia**) in Sydney are the two most established competitors to Nucleus and CMAX. Both providers offer services broadly similar to that offered by Nucleus and CMAX and frequently compete with Nucleus and CMAX for phase 1 clinical trial opportunities. However, Linear and Scientia are both significantly smaller than Nucleus and CMAX and are likely to have a more limited capacity to undertake new trials.
38. We understand that other competitors to Nucleus and CMAX listed in Table 1 may offer more specialised services targeted at specific types of trials, such as vaccine trials or oncology trials in patient populations.
39. Two new providers have recently commenced supplying phase 1 clinical trial services in Melbourne. They are Veritus with a 42 bed unit and Doherty Clinical Trials with a 25 bed unit. Novatrials has also recently commenced supplying phase 1 clinical trial services in Newcastle, NSW with a 30 bed unit. The ACCC understands that drug sponsors are risk-averse and prefer to engage providers with a proven track record of successfully undertaking phase 1 clinical trials. For this reason, it can take several years for new entrants to establish themselves as reputable trial providers and utilise their full capacity. New entrants may therefore impose a lower level of competitive constraint compared to other established providers.

The proposed acquisition

40. Blackstone, which indirectly owns Nucleus, proposes to acquire from which currently owns CMAX.

Future with and without the acquisition

41. In assessing a proposed acquisition under section 50 of the CCA, the ACCC considers the effects of the acquisition by comparing the likely future state of competition if the acquisition proceeds (the 'with' position) to the likely future state of competition if the acquisition does not proceed (the 'without' position) to determine whether the proposed acquisition is likely to have the effect of substantially lessening competition in any relevant market.
42. The ACCC considers that in the absence of the proposed acquisition, the most likely counterfactual is that CMAX would continue to operate independently.

Overlap and potential markets

43. The ACCC has not reached a concluded view on market definition.
44. The ACCC considers that alternative providers of phase 1 clinical trial services in Australia are likely to impose the strongest competitive constraint on Nucleus and CMAX. The ACCC understands that Australia is considered to be a favourable destination to conduct phase 1 clinical trials because:
 - conducting phase 1 clinical trials in Australia is generally cheaper than the US and most parts of Europe due to factors including the R&D Tax Incentive and favourable exchange rates. Market participants have indicated that it is generally 25-30% cheaper to run a clinical trial in Australia compared to other jurisdictions such as the US (depending on the exchange rate) and that it can be up to 60% cheaper including the R&D Tax Incentive.
 - Australia has a favourable regulatory environment for early-phase clinical trials owing to the CTN scheme which can allow for new trials to commence in 4-6 weeks compared to up to 6 months in the USA.
 - the quality of data produced from phase 1 clinical trial units in Australia is regarded as high and therefore is easily transferred to other jurisdictions, where drug sponsors often undertake later phase clinical trials.
 - Australia has a multicultural population which can be valuable when conducting phase 1 clinical trials, to ensure drugs are tested in diverse cohorts.
45. For the reasons listed above, customers may have a preference for using a phase 1 clinical trial service provider based in Australia. Therefore, alternative Australian phase 1 clinical trial service providers are likely to impose the most competitive constraint on Nucleus and CMAX. In particular, the ACCC understands that, based on market inquiries to date, Australian-based drug sponsors generally use Australian providers of phase 1 clinical trials. .
46. The ACCC is considering the degree of constraint likely to be imposed on the Blackstone owned providers by overseas providers of phase 1 clinical trial services. The ACCC is also considering whether the degree of competitive constraint provided by overseas based providers of phase 1 clinical trial services varies depending upon whether the drug sponsor is based in Australia or overseas.
47. The ACCC is also considering whether competition concerns may arise in relation to the supply of CRO services in Australia.

The ACCC invites comments from market participants on its preliminary views about relevant markets. In particular, market participants may wish to comment on:

- the benefits and any drawbacks associated with conducting phase 1 clinical trials in Australia compared to other jurisdictions
- the degree of competitive constraint imposed by other Australian providers of phase 1 clinical trial services, including in relation to quality, pricing and capacity to undertake new trials quickly, and
- the degree of competitive constraint provided by internationally based providers of phase 1 clinical trial services which compete with Nucleus and CMAX, including in relation to quality, pricing, capacity to undertake new trials quickly and regulatory approval systems.

Issue of concern – reduction in competition in the supply of phase 1 clinical trial services in Australia

48. The ACCC's preliminary view is that the proposed acquisition is likely to have the effect of substantially lessening competition in the supply of phase 1 clinical trial services in Australia. This is because the proposed acquisition would bring the two largest providers of phase 1 clinical trial services in Australia under common ownership and accordingly is likely to significantly lessen the competitive constraints on these providers.

Loss of a close competitive constraint

49. The ACCC's preliminary view is that Nucleus and CMAX are close competitors for the supply of phase 1 clinical trial services in Australia and that the proposed acquisition would result in the removal of an effective competitive constraint. Nucleus and CMAX are currently the two largest providers of phase 1 clinical trial services in Australia, with Nucleus holding approximately 39.4% of the phase 1 clinical trial bed capacity in Australia and CMAX holding approximately 19.9%. Nucleus and CMAX both focus on providing phase 1 clinical trial services to biotechnology and pharmaceutical companies based both in Australia and internationally.
50. The ACCC understands that drug sponsors that have determined they wish to undertake a phase 1 clinical trial in Australia will generally seek tender responses from at least Nucleus and CMAX. Smaller providers may also be considered.
51. Drug sponsors that the ACCC has spoken with have indicated that the ability of phase 1 clinical trial service providers to commence clinical trials in a short period can be important in choosing a provider. Market participants indicated that, due to Nucleus and CMAX having significantly larger bed capacity than other Australian competitors, they are likely to have increased ability to commence trials more quickly than other providers.
52. The ACCC is concerned that the proposed acquisition would remove the competitive pressure that CMAX has been exerting on Nucleus. Market participants have identified that CMAX offers high quality services at competitive prices, with most considering CMAX to be Nucleus' closest competitor. Some drug sponsors have also indicated that CMAX offers increased flexibility and the ability to commence trials and deliver results from trials more quickly than Nucleus.

53. Market participants observed that CMAX has been a driver of price competition and quality service delivery, and they expressed concern that if the proposed acquisition proceeds, there will be a loss of competitive constraint on Nucleus.

Competitive constraint from other providers

54. The ACCC is considering whether other providers of phase 1 clinical trial services could exert sufficient competitive constraint on the Blackstone owned providers if the proposed acquisition proceeds. The ACCC is considering the level of competitive constraint provided by other providers of phase 1 clinical trial services, including the constraint provided by other Australian providers and internationally based providers of phase 1 clinical trial services.
55. Other Australian providers of phase 1 clinical trial services include Linear, Scientia, University of the Sunshine Coast Clinical Trials and new entrants Veritus, Doherty Clinical Trials and Novatrials. However, these providers all have significantly less bed capacity than Nucleus and CMAX and therefore less capacity to undertake trials.
56. There are also a wide range of internationally based providers of phase 1 clinical trial service providers, in jurisdictions such as the USA, Europe, Canada and New Zealand. As outlined above at paragraph 44, the ACCC understands that there are a number of factors that make Australia a favourable destination to undertake phase 1 clinical trials. These include the relative speed at which phase 1 clinical trials can be commenced, the availability of the R&D Tax Incentive, the quality of data produced from Australian phase 1 units and the multicultural population available in Australia.
57. For these reasons, the ACCC considers that internationally based providers of phase 1 clinical trial services are likely to only provide a limited competitive constraint on the pricing and quality of phase 1 clinical trial services provided in Australia. The ACCC understands, based on market inquiries to date, that drug sponsors based in Australia prefer conducting trials in Australia including for the reasons outlined in paragraph 44.

Barriers to entry and expansion

58. The ACCC is considering whether new entry or expansion into Phase 1 clinical trials would be likely, timely and sufficient to prevent a substantial lessening of competition that might otherwise result from the proposed acquisition.
59. Market feedback suggests that barriers to commence the supply of phase 1 clinical trial services are likely to be relatively high and include:
- recruitment of qualified staff, which may be in limited supply, or adequate time to recruit, train and upskill staff coming from different backgrounds
 - building a database of interested healthy volunteers, and
 - developing a reputation and track record for quality which is required for drug sponsors to have confidence to use the services and is likely to take several years.
60. Phase 1 clinical trials involving healthy volunteers are generally conducted in purpose-built clinical trial facilities which have inpatient bed capacity. These facilities are equipped similarly to hospitals and therefore capital costs to construct an appropriate facility can be high. Market participants estimated that these costs can be between \$10-20 million.

61. New entrants need to recruit skilled staff. Staff are required to have medical skills, such as nursing qualifications and experience, but also experience with conducting clinical trials and handling potential adverse reactions to new medications. The ACCC understands that there are no specific training courses in Australia for these skills, so staff are usually trained by phase 1 trial providers. Some staff employed by phase 1 clinical trial service providers are also subject to non-compete clauses which restrict their ability to move to alternative providers.
62. Significantly, market feedback indicates that once a facility is established, there are significant timeframes involved with attracting sponsors (customers) and developing a reputation and track record for quality service delivery. Some new entrants indicated that for these reasons, in the first couple of years of operation they would be likely to undertake a limited number of trials with a focus on ensuring a high quality service is provided to sponsors. Market participants indicated that it may take 3-5 years to build up to full utilisation of facilities.
63. Given there are significant timeframes associated with setting up new facilities, recruiting specialised staff and developing a reputation for quality, the ACCC considers that the likelihood of timely and effective new entry or expansion in Australia is relatively low.

ACCC's preliminary views

64. In summary, the ACCC is concerned that the proposed acquisition brings the two largest providers of phase 1 clinical trial services in Australia under common ownership, removing an important source of competitive tension.
65. The ACCC is concerned that other providers are unlikely to significantly constrain the Blackstone owned providers post-acquisition. This is particularly so because Nucleus and CMAX have significantly larger capacity than other phase 1 clinical trial service providers in Australia and have established reputations for delivering on phase 1 clinical trials. Given the height of barriers to entry and expansion, the ACCC considers that timely and effective new entry or expansion is unlikely to constrain Nucleus and CMAX if the proposed acquisition proceeds.
66. Accordingly, the ACCC is concerned that the proposed acquisition is likely to result in lower service quality and/or higher prices for customers of phase 1 clinical trial services in Australia than would occur in a future without the proposed acquisition.

Questions for market participants

The ACCC invites comments from market participants on:

- the closeness of competition between Nucleus and CMAX
- the degree of competitive constraint imposed on Nucleus and CMAX by other providers of phase 1 clinical trial services in Australia, including Linear, Scientia, University of the Sunshine Coast Clinical Trials and new entrants Veritus, Doherty Clinical Trials and Novatrials
- the degree of competitive constraint on Nucleus and CMAX by internationally based providers of phase 1 clinical trial services
- the likelihood of new entry and/or expansion in the supply of phase 1 clinical trial services in Australia, and
- whether the degree of competitive constraint imposed on Nucleus and CMAX differs by domicile of drug sponsor, type of phase 1 clinical trial, or any other factor.

Issue that may raise concerns – foreclosure or frustration of rival CROs’ access to phase 1 clinical trial services in Australia

67. The proposed acquisition would result in Blackstone having interests in the two largest phase 1 clinical trial service providers as well as the Precision for Medicine CRO, which provides CRO services in Australia, although we understand its Australian operations are currently limited.
68. While the ACCC is concerned that, at a general level, this loss of competition between Nucleus and CMAX is likely to lead to higher prices and/or lower service levels for phase 1 clinical trials services in Australia, the ACCC is also considering Blackstone’s ability and incentive post-acquisition to foreclose rival CROs to Precision for Medicine from accessing phase 1 clinical trial services provided by Nucleus and CMAX, which may lessen competition in the provision of CRO services if those rivals cannot access competitively effective alternative providers of phase 1 clinical trial services.
69. CROs play an important role in Australia in assisting sponsors with selecting phase 1 trial sites, and overseeing trials that are being undertaken. Many international drug sponsors, particularly those with no Australian operations, also rely on CROs with operations in Australia to act as an Australian sponsor, which is a regulatory requirement to conduct clinical trials in Australia.
70. A large proportion of drug sponsors that the ACCC spoke with indicated that they would engage a CRO to assist them in selecting and liaising with phase 1 clinical trial service providers, as CROs have expertise that drug sponsors generally do not have internally.
71. The ACCC is considering the extent to which Blackstone, through its interest in Precision for Medicine and the two largest providers of phase 1 clinical trial services, would have the ability and incentive to restrict rival CROs from accessing phase 1 clinical trial services offered by Nucleus and CMAX and if such a strategy was adopted, whether it would substantially lessen competition in a market for CRO services.

Questions for market participants

The ACCC invites comments from market participants on its concerns in relation to:

- whether engagement with CROs is a critical step and the extent to which drug sponsors can and do engage with phase 1 clinical trial service providers directly
- the extent of Precision for Medicine’s operations in Australia, and
- whether rival CROs would have access to competitively effective alternative providers of phase 1 clinical trial services if they were unable to access services provided by Nucleus and CMAX.

ACCC's future steps

As noted above, the ACCC invites submissions from market participants on each of the issues identified in this Statement of Issues and on any other issue that may be relevant to the ACCC's assessment of this matter. Submissions should be emailed to mergers@accc.gov.au by no later than 21 November 2024.

The ACCC will finalise its view on this matter after it considers submissions invited by this Statement of Issues.

The ACCC intends to publicly announce its final view by 6 February 2025. However, the anticipated timeline may change in line with the *Informal Merger Review Process Guidelines*. A Public Competition Assessment explaining the ACCC's final view may be published following the ACCC's public announcement.